



Patient Name : **Mr.GANDHI**

Age / Gender : 53 / Male

UHID : 1655

Referred By : SELF



SID : 
202604 - 251

Contact No : 8008626479

Collected On : 07-04-2026 07:26 AM

Reported On : 07-04-2026 06:55 PM

FINAL REPORT

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
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BIO CHEMISTRY

BLOOD SUGAR FASTING & PP

BLOOD GLUCOSE -(FBS) (Method:Hexokinase)	98	mg/dL	Normal: 70 -100 mg/dL Impaired FG: 100-125 mg/dL Diabetes mellitus: >/=126 mg/dL
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Interpretation :

It measures the Glucose levels in the blood with a prior fasting of 9-12 hours. The test helps screen asymptomatic/ asymptomatic person who is at risk for Diabetes. It is also used for regular monitoring of glucose levels in people with Diabetes.

Reference: American Diabetes Association. Standards of Medical Care in Diabetes-2020.

BLOOD GLUCOSE, POST PRANDIAL (PP), 2 HOURS, PLASMA	133	mg/dL	80 - 140
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HEMATOLOGY

Haemoglobin (Method:Cyanide Free Lyse Hemoglobin)	12.8	gm% g/dL	Male: 13.0 - 17.0 gm% g/dL Female: 11.5 - 15.0 gm% g/dL 6yrs -12yrs:11.5-14.5 gm% g/dL
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DEPARTMENT OF CLINICAL BIOCHEMISTRY I

KFT (RENAL FUNCTION TEST),Serum

UREA (Method:Urease/UV)	19	mg/dL	12.8-42.8 mg/dL
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Interpretation :

Urea is the major nitrogen-containing metabolic product of protein and amino acid catabolism. It is increased in pre-renal uraemic conditions such as high protein diet, increased protein catabolism, Gastrointestinal hemorrhage, dehydration, heart failure, etc. post-renal uremia is seen in malignancy, nephrolithiasis and prostatism.

Reference: Tietz Fundamentals of Clinical Chemistry and Molecular Diagnostics

Serum,CREATININE (Method:Alkaline Picrate)	0.7	mg/dL	Male : 0.60-1.30 mg/dL Female : 0.55-1.02 mg/dL Child : 5 to <12 years :0.44-0.65 : 12 to <15 years : 0.52-0.69
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Interpretation :

Home Collection Available



Near by SBI Bank, SS Hospital Tada,
Tirupati Dist, A.P - 524 401.

CONDITION OF REPORTING

1. The Laboratory results given are for the information and interpretation of the referring Clinicianonly.
2. The lab reports should not be used in isolation to make a diagnosis or change treatment plans, but should be interpreted along with other relevant clinical and imaging details.
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Contact No : 8008626479

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TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
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Creatinine is a nitrogenous waste product produced by muscles from creatinine. Creatinine is majorly filtered from the blood by the kidneys and released into the urine, so serum creatinine levels are usually a good indicator of kidney function. Serum creatinine is more specific and more sensitive indicator of renal function as compared to BUN because it is produced from muscle at a constant rate and its level in blood is not affected by protein catabolism or other exogenous products. It is also not reabsorbed and very little is secreted by tubules making it a reliable marker. Serum creatinine levels are increased in pre renal, renal and post renal azotemia, active acromegaly and gigantism. Decreased serum creatinine levels are seen in pregnancy and increasing age.

Uric Acid (Method:Uricase)	4.1	mg/dL	Male : 3.5-7.2 mg/dl Female : 2.6-6.0 mg/dl
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Interpretation :

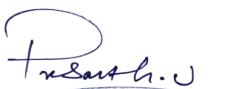
It is the major product of purine catabolism. Hyperuricemia can result due to increased formation or decreased excretion of uric acid which can be due to several causes like metabolic disorders, psoriasis, tissue hypoxia, pre-eclampsia, alcohol, lead poisoning, acute or chronic kidney disease, etc. Hypouricemia may be seen in severe hepato cellular disease and defective renal tubular reabsorption of uric acid.

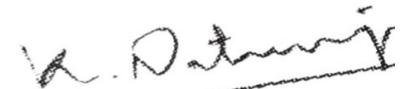
SODIUM (Method:ISE Direct)	140	mmol/L	136-149 mmol/L
Pottasium (Method:ISE Indirect)	3.78	mmol/L	3.5-5.3 mmol/L
Chloride (Method:ISE Direct)	101	mmol/L	98-108 mmol/L

Interpretation :

Electrolyte profile is the determination of body fluid concentrations of the four major electrolytes (sodium, potassium, chloride and bicarbonate). Serum electrolytes have a role in water homeostasis, acid & base balance, muscle function, etc. Abnormal electrolyte concentrations may be the cause or consequence of several medical disorders and require clinical correlation.

Disclaimer:Test results released pertain to the specimen submitted. All test result are dependent on the quality of the sample received by the laboratory. Test result may show interlaboratory variations. Laboratory investigation are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician.


Verified By


Dr. Prashanth

Home Collection Available



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TEST NAME

RESULT

UNIT

BIOLOGICAL REFERENCE INTERVAL

U.Prashanth B.sc,MLT.,

Dr.K Natrajan.M.sc,M.phil.,Ph.D
Consultant BioChemist

End of Report

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